

PROGRAM EVALUATION

H.Y.P.E. to Empower

HIV Health & Linkage Services
Gwinnett, Newton and Rockdale Counties
June 2026

Prepared by: Lidya Kassahun
HIV Prevention Program Coordinator & Analyst Intern

PART ONE: PROGRAM EVALUATION PLAN

1.1 Introduction

The Health & Linkage Services (HLS) Program is a community-based HIV prevention initiative implemented by H.Y.P.E. to Empower, Inc. throughout Metropolitan Atlanta. The program was developed to address HIV-related disparities among populations disproportionately affected by HIV, including Black/African American communities, Hispanic/Latino communities, LGBTQ+ populations, women of color, and young adults.

The program provides HIV prevention education, rapid HIV testing, PrEP education and navigation, linkage-to-care services, community outreach, and referrals to supportive services. Through partnerships with community organizations, healthcare providers, and public health agencies, H.Y.P.E. seeks to reduce barriers to HIV prevention and improve access to culturally responsive health services.

The purpose of this evaluation plan is to provide a systematic framework for assessing program implementation, effectiveness, and impact. The evaluation is guided by the Centers for Disease Control and Prevention (CDC) Framework for Program Evaluation in Public Health and is designed to support continuous quality improvement, stakeholder decision-making, and future program sustainability.

Table 1. Program Description

Component	Description
Program Name	Health & Linkage Services Program
Organization	H.Y.P.E. to Empower, Inc.
Service Area	Gwinnett, Newton, and Rockdale, Georgia
Target Population	Black/African American communities, Hispanic/Latino communities, LGBTQ+ populations, women of color, young adults, and individuals at risk for HIV
Program Purpose	To reduce HIV-related disparities through HIV testing, prevention education, PrEP navigation, linkage-to-care services, and community outreach
Evaluation Type	Mixed-Methods Process and Outcome Evaluation
Evaluation Period	March – June 2026

1.2 Engage Stakeholders

Stakeholder engagement ensures that evaluation activities reflect community priorities, organizational needs, and program goals. Stakeholders contribute to program implementation, data collection, service delivery, community outreach, funding, and utilization of evaluation findings.

Internal and external stakeholders were identified based on their involvement in program implementation, funding, service delivery, referral networks, and community engagement activities.

Figure 1. Stakeholder Engagement Framework

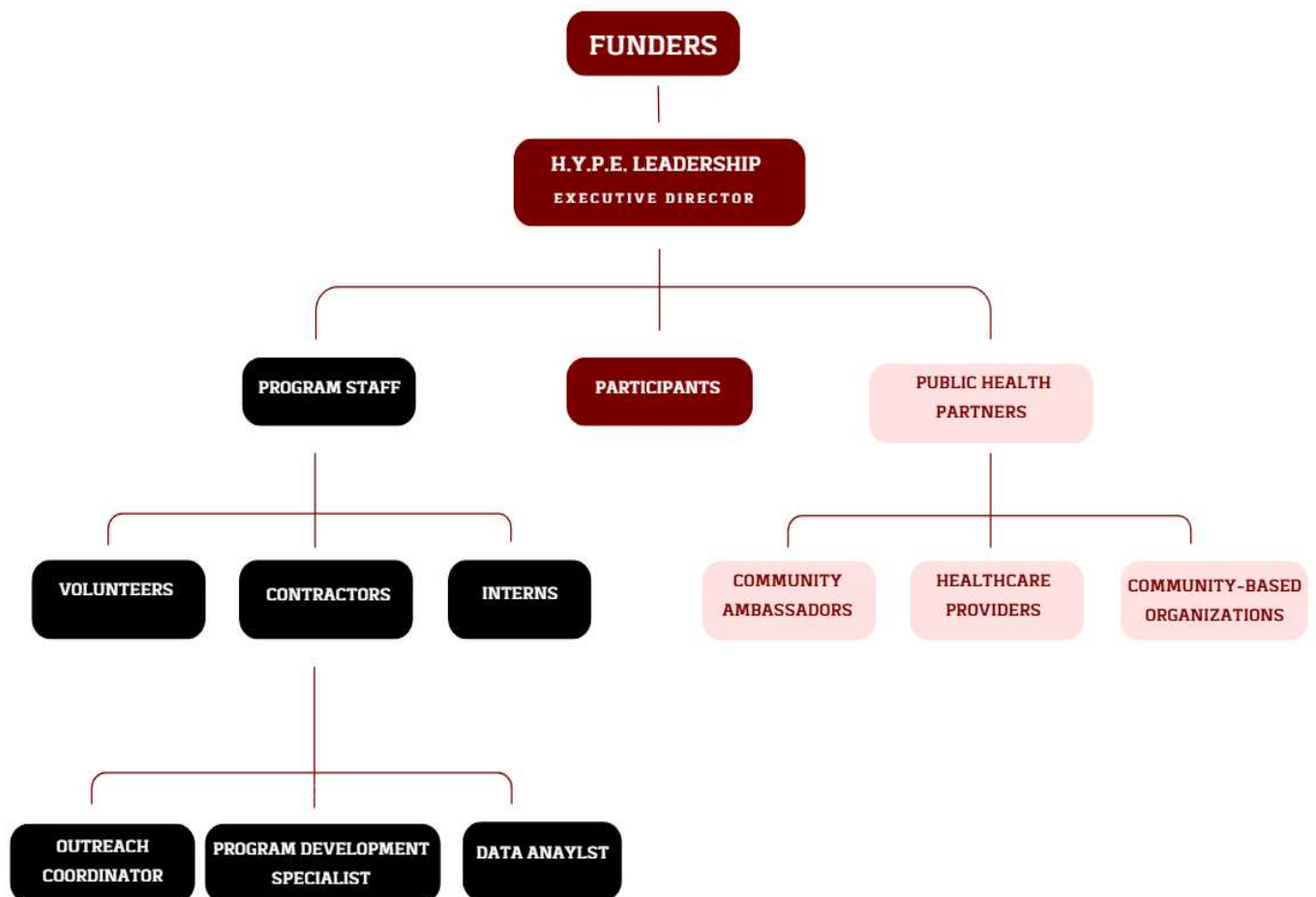


Table 2. Stakeholder Analysis

Stakeholder	Role in Program	Role in Evaluation
Executive Director	Program oversight	Uses findings for planning and decision-making
Program Staff	Service delivery	Collect program data
Program Evaluator / Data Analyst	Evaluation design and reporting	Conduct analysis and prepare reports
Community Ambassadors	Outreach and engagement	Provide participant feedback
Intern	Evaluation support	Data analysis and reporting
Healthcare Providers	Referral and linkage-to-care services	Provide referral outcome information
Participants	Receive services	Provide survey feedback
Community-Based Organizations	Outreach and referral support	Partner feedback and collaboration
Funders	Financial support	Review outcomes

1.3 Describe the Program

The Community Needs Assessment identified multiple HIV prevention and care gaps within Gwinnett, Newton & Rockdale Counties. Key findings included low levels of PrEP awareness, HIV testing gaps, persistent racial and ethnic disparities, and barriers to healthcare access.

Table 3. Community Needs Assessment Summary

Need	Evidence
Low PrEP Awareness	Only 31.8% of assessed participants had heard of PrEP
HIV Testing Gaps	21.7% had never tested; 25.0% had not tested in over 12 months
Elevated HIV Burden	Gwinnett diagnosis rate exceeded Georgia state averages
Racial/Ethnic Disparities	Black/African American prevalence rate 928.6 per 100,000; Hispanic/Latino 559.7 per 100,000

Access Barriers	Transportation, healthcare access, stigma, and language barriers identified
-----------------	---

Theory of Change

If H.Y.P.E. provides culturally responsive HIV education, testing, PrEP navigation, and linkage services, then participants will increase HIV knowledge, access prevention services, engage in care, and adopt healthier prevention behaviors, resulting in reduced HIV disparities and improved community health outcomes.

Figure 2. HLS Program Evaluation Gantt Chart

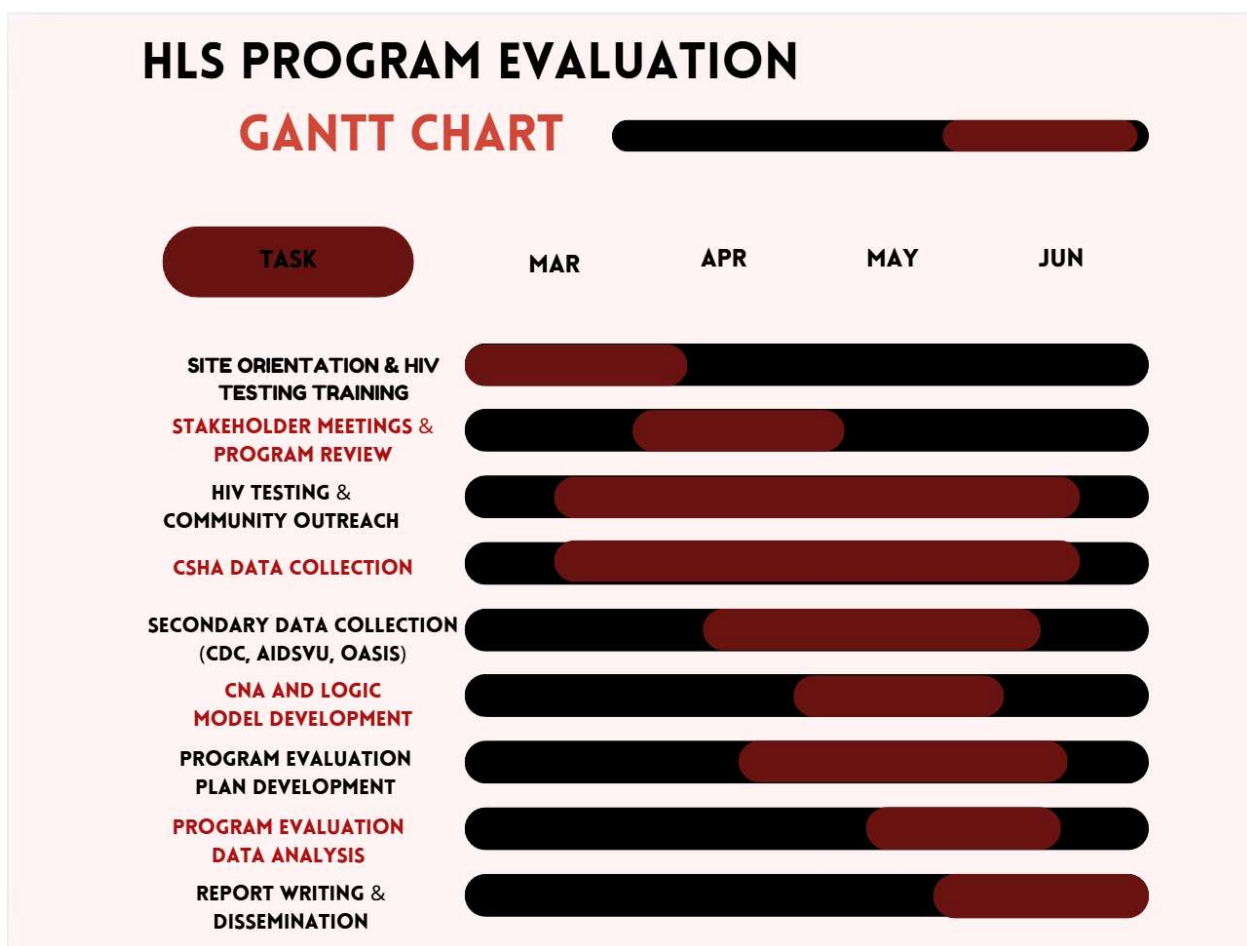


Figure 3. HLS Program Logic Model: SMART Objectives, Inputs, Activities, Outputs, and Outcomes

SMART Objectives	Inputs	Activities	Outputs	Short-Term Outcomes	Intermediate Outcomes	Long-Term Outcomes
Knowledge: $\geq 80\%$ demonstrate increased HIV prevention knowledge (pre/post assessment) by program end.	Staff (Program Coordinator, Health Educators, HIV Test Counselors, PrEP Navigators, Peer Educators)	Provide HIV testing and counseling services	Number of individuals tested	Increased HIV knowledge and awareness	Increased utilization of HIV prevention and support services	Reduced HIV transmission rates
Testing: $\geq 75\%$ of enrolled participants receive an HIV test during participation.	Funding (grants, public health funding, community support)	Conduct community outreach events and awareness campaigns	Number of individuals reached through outreach	Increased awareness of HIV prevention, testing, and PrEP	Increased PrEP uptake and adherence among eligible participants	Reduced HIV-related disparities among underserved populations
PrEP Navigation: $\geq 60\%$ of eligible participants connected to PrEP services within 3 months of enrollment.	Partnerships (clinics, health departments, community-based organizations, schools)	Provide referrals and linkage-to-care services	Number of referrals made and completed	Reduced HIV-related stigma and misconceptions	Improved linkage-to-care and retention in care	Improved health equity and access to healthcare
Linkage-to-Care: $\geq 90\%$ of participants with reactive results linked to medical care within 30 days.	Educational Materials (brochures, social media content, workshops, presentations)	Deliver culturally responsive HIV education sessions	Number of participants receiving education	Increased adoption of HIV prevention behaviors (condom use, routine testing, PrEP awareness)	Improved care coordination and continuity of services	Sustainable HIV prevention systems and healthier communities

Behavior Change: $\geq 70\%$ report adopting ≥ 1 HIV prevention behavior within 6 months of completion.	Testing Supplies & Technology (rapid test kits, data systems, outreach tools)	Facilitate PrEP navigation and support services	Number of participants connected to PrEP services	Increased confidence in accessing HIV-related services	Increased engagement in preventive healthcare	Reduced incidence of new HIV infections
	Community Resources (support groups, mental health services, case management)	Provide follow-up support and case management	Number of follow-up contacts and support services provided	Increased trust in healthcare systems and community resources	Improved participant health outcomes and self-management	Enhanced community well-being and quality of life

1.4 Focus the Evaluation Design

Evaluation Purpose

The evaluation seeks to:

- Assess implementation fidelity.
- Determine whether priority populations are being reached.
- Evaluate program outcomes.
- Identify opportunities for quality improvement.
- Support future funding and sustainability efforts.

Table 4. Evaluation Questions

Domain	Evaluation Question
Reach	Are priority populations being reached?
Testing	Is testing reaching individuals at increased risk?
PrEP	Is awareness and utilization improving?
Linkage	Are referrals being successfully completed?
Engagement	Are community partnerships strengthening outreach?

1.5 Gather Credible Evidence

Table 5. Evaluation Data Sources

Data Source	Purpose
HIV Testing Logs	Testing outcomes and participant demographics
Outreach Records	Program activity monitoring
Referral Tracking Forms	PrEP referrals and service connections
Community Sexual Health Assessment	Community needs identification
Post-Event Survey	Knowledge and satisfaction outcomes
Program Records	Service outputs and performance indicators

Table 6. Process Indicators

Indicator	Data Source	Frequency
HIV Tests Conducted	Testing Logs	Monthly
Outreach Events Conducted	Event Records	Monthly
Participants Reached	Attendance Records	Monthly
Condoms Distributed	Program Records	Monthly
PrEP Referrals	Referral Tracking Forms	Monthly

Table 7. Outcome Indicators

Outcome	Indicator	Data Source
HIV Prevention Knowledge	Knowledge Score	Post-Event Survey
PrEP Awareness	Awareness Measure	Community Assessment
HIV Testing Access	Number Tested	Testing Logs
Participant Satisfaction	Satisfaction Rating	Post-Event Survey
Service Need Identification	Requested Services	Community Assessment
Priority Population Reach	Demographic Distribution	Program Records

1.6 Data Analysis Plan

This evaluation utilized a mixed-methods approach by integrating quantitative program performance data and survey results with qualitative participant feedback to provide a more comprehensive assessment of program implementation and outcomes.

Quantitative Analysis

Data analyzed in this evaluation were drawn from two primary sources: (1) program administrative records collected by H.Y.P.E. staff during the evaluation period (March–June 2026), and (2) the Community Needs Assessment, which utilized publicly available HIV surveillance data from CDC AtlasPlus, AIDSVu, and the Georgia Department of Public Health. The Community Needs Assessment documented that Gwinnett County reported 3,976 people living with HIV (prevalence rate: 489.8 per 100,000) and 235 new HIV diagnoses in 2023 (rate: 29.0 per 100,000). Racial and ethnic disparities were pronounced: Black/African American residents experienced an HIV prevalence rate of 928.6 per 100,000, and Hispanic/Latino residents experienced a rate of 559.7 per 100,000. Linkage to HIV medical care stood at 82.8% in 2022, below the Ending the HIV Epidemic target of 95%, and PrEP coverage among eligible individuals was 29.8% in 2023. These community-level findings provided the epidemiological context against which program performance data were interpreted.

Quantitative data were analyzed using descriptive statistical methods, including:

- Frequencies and percentages
- Means and trend analysis
- Cross-tabulation of demographic and program indicators

Variables analyzed included:

- HIV testing volume and participant demographics
- Priority population reach
- Condom distribution and PrEP referrals
- HIV prevention knowledge and participant satisfaction
- Service needs identified through the community assessment

Qualitative Analysis

Open-ended survey responses and participant feedback were reviewed using thematic analysis.

Responses were coded to identify common themes related to:

- Program strengths
- Participant experiences
- Barriers to HIV prevention services
- Community needs and recommendations for future programming

1.7 Justify Conclusions

Evaluation conclusions were based on the triangulation of multiple data sources, including program records, participant surveys, and community assessment findings. Program success was assessed using the following criteria:

Reach

- Number of individuals tested
- Representation of priority populations
- Community engagement activities

Service Delivery

- Completion of outreach activities
- Distribution of prevention resources

- Delivery of HIV prevention education

Outcomes

- HIV prevention knowledge
- Participant satisfaction
- PrEP awareness and service utilization

Community Impact

- Identification of service gaps
- Alignment with community needs
- Progress toward reducing barriers to HIV prevention services

1.8 Dissemination Plan

Evaluation findings will be shared with internal and external stakeholders to support decision-making, quality improvement, accountability, and future program planning.

Table 8. Dissemination Plan

Audience	Product
H.Y.P.E. Leadership	Full Evaluation Report
Program Staff	Internal Presentation
Community Partners	Executive Summary
Funders	Evaluation Brief
Public Health Agencies	Summary Report
Community Members	Fact Sheet and Infographic

1.9 Continuous Quality Improvement

Evaluation findings will be used to support continuous quality improvement activities within the Health & Linkage Services Program. CQI efforts will focus on:

- Improving participant tracking systems
- Enhancing referral and follow-up procedures
- Expanding PrEP education and navigation services
- Strengthening community partnerships
- Improving data collection and reporting systems
- Increasing outreach to underserved populations

Evaluation findings will be reviewed regularly to monitor progress and guide future program modifications.